

Angioedema and neutropenia in two patients receiving domitrelle: a case series

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Abstract

We describe two unrelated patients who developed serious adverse reactions during treatment with domitrelle and a concomitant agent. The first developed agranulocytosis eight weeks into therapy; the second developed angioedema followed by persistent dizziness. Both required hospital assessment and both recovered following withdrawal. The cases are reported separately because the reactions, the time to onset and the outcomes differ materially.

Keywords: domitrelle, angioedema, agranulocytosis, case series

Introduction

Case series remain a useful early signal-detection tool where individual reports are too sparse to support formal disproportionality analysis. The two patients described here presented to different centres within a six-month period. Neither had a documented history of drug allergy. We present each case in turn, followed by a combined discussion.

Case Series

Case 1

We report a 34-year-old man with a history of no significant past medical history, who was referred to our department for assessment.

The patient had been prescribed Domitrelle 2 mg twice daily (domitrelle succinate, gastro-resistant capsule) for generalised anxiety disorder, commencing 2 February 2026. The route of administration was oral.

A 34-year-old man developed a neutrophil count of $0.3 \times 10^9/L$ with fever and mouth ulceration, beginning 29 March 2026. The reported term was agranulocytosis.

The event met the regulatory criteria for a serious adverse reaction (life threatening).

The suspected medicine was withdrawn. At the time of writing the outcome was recorded as recovering.

A 34-year-old man taking Domitrelle 2 mg twice daily presented with fever and mouth ulceration eight weeks into treatment. The neutrophil count was $0.3 \times 10^9/L$. He was managed with granulocyte colony-stimulating factor and the count is recovering. Domitrelle was permanently discontinued.

Case 2

We report a 45-year-old male with a history of asthma since childhood, who was referred to our department for assessment.

The patient had been prescribed Pralexin 100 micrograms/dose two puffs as required (pralexin, pressurised inhalation) for asthma maintenance, commencing 5 May 2026. The route of administration was inhalation.

A 45-year-old male developed swelling of the lips and tongue with difficulty swallowing, beginning 21 May 2026. The reported term was angioedema.

The event met the regulatory criteria for a serious adverse reaction (life threatening).

The suspected medicine was withdrawn. At the time of writing the outcome was recorded as recovered.

A 45-year-old male with lifelong asthma used Pralexin two puffs as required from 5 May 2026. He was additionally given Astelvia 1 g three times daily on 18 May for post-operative prophylaxis. On 21 May he developed swelling of the lips and tongue with difficulty swallowing, and on the following day persistent dizziness. He attended the emergency department and was treated with intramuscular adrenaline. Both medicines were stopped.

Discussion

The two presentations share a common suspect agent but differ in mechanism. Agranulocytosis is likely idiosyncratic and immune-mediated, whereas the angioedema in the second patient may reflect an interaction with the concomitant infusion. Neither patient was rechallenged. Reporting both together should not be read as implying a shared mechanism; they are grouped here only because the suspect agent is common to both.

Conclusion

Two serious reactions in a six-month period, in patients managed at different centres, are sufficient to justify continued monitoring. Each case should be handled as a separate individual case safety report.

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Conflict of interest

The authors declare that they have no competing interests.

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